June 16, 2025

Centers for Medicare & Medicaid Services,
Department of Health and Human Services,
File Code: CMS-0042-NC
Submitted electronically via regulations.gov

Re: CMS-0042-NC

To Whom It May Concern:

The National Council for Prescription Drug Programs (NCPDP) is a not-for-profit American National Standards Institute (ANSI) Accredited Standards Developer (ASD) consisting of more than 1,300 members representing entities including, but not limited to, claims processors, data management and analysis vendors, federal and state government agencies, insurers, intermediaries, pharmaceutical manufacturers, pharmacies, pharmacy benefit managers, professional services organizations, software and system vendors and other parties interested in electronic standardization within the pharmacy services sector of the healthcare industry. NCPDP provides a forum wherein our diverse membership can develop business solutions, including ANSI-accredited standards and guidance for promoting information exchanges related to medications, supplies and services within the healthcare system.

NCPDP standards have been named in federal legislation and regulations including, but not limited to, the Health Insurance Portability and Accountability Act (HIPAA), Medicare Modernization Act (MMA), the Health Information Technology for Economic and Clinical Health Act (HITECH) and Meaningful Use (MU). In addition to our standards work, NCPDP continues to recognize and support the role and value of pharmacists as trusted healthcare professionals. NCPDP has a strategic focus on improving interoperability between pharmacy and other healthcare constituents for pharmacy-based clinical services and care coordination. Through collaboration with other Standards Development Organizations (SDO) and industry partnerships, NCPDP is working to develop guidelines and standards to promote interoperability.

NCPDP supports the objectives and strategies put forth by the Department of Health and Human Services (HHS) within the Request for Information (RFI) on Health Technology Ecosystem as it pertains to CMS-0042-NC. As a standards organization, NCPDP's purpose is to standardize the exchange of healthcare information to improve outcomes, which directly aligns with HHS's objectives. To answer questions within this RFI, NCPDP has organized our comments around key NCPDP standards, use cases and implementation experience to align with the Centers for Medicare & Medicaid Servies (CMS) goals for improved interoperability, patient access and innovation. NCPDP aims to demonstrate how pharmacy standards can play a critical and scalable role in a modern digital health ecosystem.

Supporting the Role of Pharmacists in Care Coordination and Innovation (TD-2, TD-13, PC-1, PC-5)
In addition to our core standards development work, NCPDP continues to champion the role and value of pharmacists as trusted healthcare professionals. In 2023, NCPDP launched a new work group, *WG20: Coordination of Care & Innovation* to drive interoperability and enhance the pharmacist's role in care coordination and advancing innovation in the market. This initiative builds on our strategic emphasis on pharmacist's contributions to value-based care (VBC), chronic disease management and closing gaps in care.

NCPDP is also prioritizing improved interoperability between the pharmacy and other healthcare stakeholders to support pharmacy-based clinical services. In collaboration with other SDOs and industry partners, NCPDP is developing guidelines and standards to promote seamless clinical data exchange.

Recent resources released by NCPDP include:

- An *Executive Summary Status of Adoption – HL7®/NCPDP Pharmacist eCare Plan* (PeCP) highlighting how the PeCP can be leveraged by providers, health systems and health plans to support VBC initiatives[1].
- *Industry Guidance for Implementation of Admit, Discharge and Transfer (ADT) notifications for Pharmacy*[2] to support pharmacists' engagement in care transitions and care coordination.

Finally, NCPDP acknowledges the industry's shift toward modern standards and technologies, which is why NCPDP is increasingly focused on Application Programming Interfaces (API) for streamlined data exchange, aligning our work with federal initiatives such as the Trusted Exchange Framework and Common Agreement (TEFCA) and the CMS Interoperability Rule.

API Enablement Across Standards (TD-2 (d))

It is important to clarify "API" in this context should not be interpreted as exclusive to FHIR®-based APIs. NCPDP standards also support API-based transactions and NCPDP is in active development of new APIs, such as the Pharmacy Product Availability API. This initiative aims to offer providers and eventually patients with real-time access to medication availability across pharmacy locations, supporting transparency and continuity of care. NCPDP recommends CMS support a multi-standard API strategy to include FHIR and non-FHIR APIs, allowing for broader innovation across domains, like pharmacy, where alternative standards are already in production. NCPDP has and does support APIs and our published Connectivity Rule outlines the use of APIs for multiple standards.

Admit, Discharge and Transfer (ADT) Notifications for Pharmacies (TD-6, TD-13)

The RFI raises several questions about VBC, technical limitations and the types of data essential for provider participation. NCPDP believes ADT notifications for pharmacy represent one such essential data type. To support adoption, NCPDP recently released industry guidance on implementing ADT notifications within pharmacy workflows.

The integration of ADT notifications into pharmacy workflows will represent a significant advancement in enhancing patient care and operational efficiency. Pharmacies, as vital components of the healthcare continuum, are positioned to leverage real-time ADT notifications to improve medication management, medication reconciliation, patient education and continuity of care.

CMS currently has a mandate, specifically the *CMS Interoperability and Patient Access Final Rule (CMS-9115-F)*[3], which requires Medicare and Medicaid participating hospitals, encompassing psychiatric and critical access facilities, to transmit electronic patient event notifications, such as ADT information, to designated providers on record. These providers encompass primary care providers, post-acute care facilities and specialists. This mandate aims to enhance care coordination and ensure timely communication between healthcare entities, ultimately improving patient outcomes and experience within the healthcare system. NCPDP supports the exchange of ADT information including the sending of ADT information to consenting/participating pharmacies.

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[1] https://www.ncpdp.org/NCPDP/media/pdf/ExecutiveSummary_PeCP.pdf

[2] https://www.ncpdp.org/NCPDP/media/pdf/Industry-Guidance-for-Implementation-of-ADT-Notifications-for-Pharmacy-Version-10.pdf?ext=.pdf

[3] https://www.cms.gov/files/document/cms-9115-f.pdf

Real-Time Prescription Benefit (RTPB) Standard© (PC-1, PC-5, PC-8, TD-13)
NCPDP's RTPB Standard©, was piloted in 2019 and Version 13 is adopted in the Final Rule - CMS–4205–F2[4]; Medicare Part D and the Assistant Secretary for Technology Policy/Office of National Coordinator (ASTP).

The RTPB Standard enables the real-time exchange of prescription benefit information between a provider (physician or pharmacist) and a PBM/processor. Such information includes:

- Patient eligibility
- Product coverage
- Estimated patient pay for a chosen product and pharmacy
- Coverage restrictions and alternatives when they exist
- Information as close as possible to what the patient may experience at the pharmacy

Consumer-facing Real-Time Prescription Benefit Check (RTPBC)
NCPDP supports the CARIN Alliance and HL7®'s development of a FHIR®-based, Consumer Real-Time Pharmacy Benefit Check (RTPBC) Implementation Guide (IG), which is already in use within applications to enable patients to access personalized drug pricing and coverage information.

Importantly, the CARIN Alliance consumer-facing IG is being actively harmonized with the latest version of NCPDP's prescriber-facing RTPB Standard Version 13©. This alignment is critical because RTPB data is not sourced from a single payer. Instead, consumer and provider-facing applications must integrate with multiple payers and PBMs, each potentially using different configurations or data models.

By harmonizing these two standards, the ecosystem ensures:

- Data consistency across patient and provider-facing tools, regardless of which payer is queried
- Reduced integration complexity for health plans and application developers, using shared data structures and profiles
- Scalable, standards-based transparency tools to improve the patient experience and clinical decision-making

NCPDP recommends CMS and ASTP:

- Formally recognize the alignment between the CARIN Alliance HL7 FHIR based IG and NCPDP RTPB Standard Version 13© as a strategic interoperability framework for prescriber and consumer access
- Support testing, certification and implementation efforts to encourage health plans and PBMs to conform to these standards via a RTPB API interface
- Incorporate RTPB alignment guidance into future updates to API certification, TEFCA use case development or Standards Version Advancement Process (SVAP)

A unified, multi-payer RTPB framework to span both patient and provider access is critical to achieving CMS's vision for cost transparency, patient empowerment and equitable access to prescription benefits.

Patient access and patient transparency are two important areas where CMS and ASTP have and can continue to make an impact around healthcare technology.

[4] https://public-inspection.federalregister.gov/2024-12842.pdf?utm_campaign=pi+subscription+mailing+list&utm_medium=email&utm_source=federalregister.gov

Digital Therapeutics (PC-2, TD-3)
For the past several years, NCPDP's Digital Therapeutics Task Group has focused on integrating prescription digital therapeutics (DTx) into the healthcare system by adapting existing NCPDP industry standards.

In 2024, the NCPDP Foundation funded research to identify pain points, barriers and opportunities to use existing standards for coverage and reimbursement of these therapeutics. The research showed how four NCPDP standards were identified as utilized within the payment workflow. The Formulary & Benefit, Real-Time Prescription Benefit, SCRIPT and Telecommunication Standards could be used to support prescribing, dispensing and reimbursement.

In July 2025, NCPDP will be hosting a stakeholder action group with the following discussion topics:

- Mechanisms to support the standardized exchange of patient activation codes between digital therapeutic developers and dispensers (pharmacies, prescribers or other authorized dispensers)
- Approaches to enable the exchange of standardized utilization data from developers to providers and/or payers

NCPDP recommends CMS and ASTP work with NCPDP to organize and develop potential policies to support DTx in workflows.

Patient Matching/Identity and Facilitator Model (PC-3, PC-8, TD-13)
Patient matching is a critical component of the entire interoperability process. Current demographic-only methodologies used to match patients are not sufficient to ensure accurate patient matching. *Matching patients to their medical records can be as low as 80% accurate within a single care setting and drops to 50% accuracy among organizations that share electronic health information*[5]. *On average, healthcare organizations report spending $1.3M annually on patient identity resolution*[6].

NCPDP believes accurately identifying and matching a patient is essential for healthcare to ensure efficient and safe patient management and care.

NCPDP has a patient matching solution to accurately manage patient identification across the healthcare ecosystem. The Patient Experience Identifier (PEI) Solution leverages NCPDP's Universal Patient Identifier (UPI) currently incorporated into NCPDP standards, uses Experian's expansive consumer demographic information and AI matching methodologies to identify record matches and duplicates in a patient roster file and then assign a unique UI to each patient in the file.

The NCPDP Facilitator Model is a solution to help facilitate the exchange of public health records coordinated in an efficient manner. This model can support patient facing and other productions of digital solutions being offered in the marketplace.

In addition, NCPDP, in collaboration with STChealth, has developed and implemented a non-jurisdictional infrastructure to store immunization information and tested the model with pharmacy pilot participants using COVID-19 immunization data. The pilot participants also utilized the NCPDP PEI to match patient records across 57 distinct pharmacy locations.

[5] https://www.statnews.com/2022/06/22/national-patient-id-essential-patient-safety/
[6] https://patientidnow.org/wp-content/uploads/2022/11/PIDN-Research-Findings-Final.pdf

The results of the pilot show comprehensive access to immunization data by all care team members, including pharmacists, is critical in removing silos across jurisdictions, reduces provider burden and improves access to care.

Consent Management (TD-19)
NCPDP has worked to support the processes for obtaining patient consent to be shared between providers. The Specialty Medication Enrollment FHIR® IG (developed jointly by NCPDP and HL7®) leverages FHIR to enable the request to obtain consent from the patient and subsequent exchange of consent information to expedite access to care. NCPDP is collaborating with the HL7 FHIR at Scale Taskforce (FAST) to improve these consent processes, which are fundamental to patient care.

United States Core Data for Interoperability (USCDI) (TD-7, VB-4)
The use of the USCDI has improved healthcare interoperability, but adoption is still in the early stages. The existing USCDI data elements do not currently contain the full extent of the data elements needed for pharmacy services and prescription drug benefit processing. Additional data elements are needed in USCDI to add value to those practicing at the community pharmacy level.

It also appears USCDI has a significant amount of duplication of data elements and many of the data elements may not have clinical value. Currently, data is included to exchange information for medical claims, but there is some missing information useful for pharmacies participating in VBC arrangements. NCPDP will continue to review USCDI versions, providing comments and recommendations where needed and request additional data elements as applicable.

TEFCA (PC-10, PR-6)
As pharmacists play an increasingly important role in patient care and expand their scope of services to include more clinical patient care, pharmacists providing these services will need to obtain a more complete picture of their patients' health records. Pharmacists delivering this clinical care understand the need to participate in TEFCA and are recognizing the importance of utilizing systems built on electronic health record (EHR) platforms to support comprehensive care delivery, along with traditional, pharmacy-specific technology and standards. Community, long-term care and specialty pharmacies have not adopted patient EHR technology and incentives or reimbursement models to incentivize pharmacists, and pharmacy provider participation are not in place.

The lack of patient EHR technology for pharmacists poses a barrier to participation in TEFCA, resulting in a gap in patient clinical data available to all providers, including pharmacists.

NCPDP recommends HHS:

- Support policies and programs to enhance pharmacist scope of practice and provider status
- Clarify how TEFCA will support pharmacy, provider and patient access to comprehensive patient medication history, especially when it resides outside EHR systems
- Consider financial incentives or assistance for pharmacies and pharmacists adopting EHR technology

The lack of a defined pathway for pharmacy participation in TEFCA and other federally supported exchange networks raises unique concerns about information blocking. Pharmacists are designated as "actors" under the 21st Century Cures Act and are subject to information blocking prohibitions. However, they often lack the technical

infrastructure, certified EHRs or network access needed to fulfill these obligations, especially when data is held in TEFCA networks or EHR-based platforms.

NCPDP urges HHS to:

- Consider policies to emphasize education and technical assistance before imposing penalties on healthcare providers
- Refrain from imposing penalties in an initial instance on healthcare providers, such as pharmacies and pharmacists who are not eligible for incentive payments
- Prioritize education and technical assistance for healthcare providers, such as pharmacies and pharmacists not eligible for incentive payments
- Collaborate with pharmacies toward mutual goals in improving healthcare and leveraging the initial work of the Health Information Technology Advisory Committee (HITAC) Pharmacy Interoperability and Emerging Therapeutics Task Force

Standardized Medication Profile (PC-1, PC-3, PC-8)

The communication of health information, including medications, is critical to ensuring safe and effective transitions from one healthcare setting to another. Medication errors, poor communication and poor coordination between providers, along with the rising incidence of preventable adverse events and hospital re-admissions, have drawn national attention to the importance of the timely transfer of medication information between post-acute care (PAC) providers, including long-term care hospitals (LTCH), in-patient rehabilitation facilities (IRF), skilled nursing facilities (SNF), home health agencies (HHA) and other settings.

About 40% of patients discharged from acute care receive PAC services[7], and in 2018 Medicare fee-for-service spending on PAC expenditures was $58.6 billion[8]. However, little is known about the transfer of health information, such as medications, during transitions of care, including the types of medication information transferred or the mode(s) of transfer used.

The Standardized Medication Profile is a joint effort between HL7® and NCPDP supported by the PACIO project[9]. An HL7 FHIR® IG was co-developed to support this transition of care by providing information on the medications the patient is receiving through their journey.

Patients in PAC settings are typically complex from a healthcare perspective and have many stops during their healthcare journey, but the IG is intended to cover the FHIR-based exchange of medication information compiled and exchanged between healthcare settings, pharmacies and home- and community-based services (HCBS) broadly.

Remote Patient Monitoring (RPM) (TD-3)

RPM is an emerging process using digital health products to connect providers with patients and pharmacists, who are at the forefront of this technology. Pharmacy software supporting RPM should function like a smart assistant, not just a prescription adjudication tool. It should connect the dots among patients, pharmacists and providers with no duplicate entry.

[7] https://build.fhir.org/ig/HL7/smp-ig/usecases.html#fn:1
[8] https://build.fhir.org/ig/HL7/smp-ig/usecases.html#fn:2
[9] https://build.fhir.org/ig/HL7/fhir-pacio-pfe/index.html

NCPDP recommends HHS collaborate with the industry to standardize RPM within the pharmacy sector:

- RPM platforms to integrate with NCPDP-aligned workflows
- Pharmacy systems to utilize appointment/scheduling APIs
- Help drive adoption and interoperability in the field
- Help standardize a way for two-way communication with clinics

Pharmacist eCare Plan (PeCP) and VBC (TD-13)

PeCP exemplifies interoperability standards through collaboration between HL7® and NCPDP. It facilitates seamless, medication-related care plan exchange and provider communication as a component of VBC agreements, capturing patient health concerns, medication activities, plans, pharmacist interventions and goals. PeCP supports pharmacist integration into healthcare teams, utilizing HL7 CDA (Clinical Document Architecture) and FHIR® standards for comprehensive medication therapy management and care coordination.

In 2023, HITAC's Pharmacy Interoperability and Emerging Therapeutics Task Force identified PeCP as a key solution for advancing pharmacy care delivery. Through their comprehensive review, the task force generated 34 recommendations which were submitted to ASTP supporting enhanced pharmacy interoperability. Throughout the recommendations, PeCP was highlighted as a central component for standardized care plan exchange.

Pharmacists require sophisticated tools to help support comprehensive patient care documentation, seamless information exchange and collaborative care delivery. Building on this growing need, NCPDP will expand upon foundational work completed in 2012 with HL7 on Pharmacy/Pharmacist Functional Profiles. These profiles establish core EHR building blocks and create essential pathways to full PeCP implementation. The vision encompasses seamless interconnection between pharmacy management systems and medical EHRs, creating a unified platform for care team collaboration. While some aspects remain in conceptual stages, NCPDP continues advancing this framework to achieve comprehensive system integration, ultimately supporting improved patient outcomes through enhanced care coordination. This work represents a crucial step toward realizing the full potential of pharmacy-based patient care within the broader healthcare ecosystem.

Summary

NCPDP has numerous standards and strategic initiatives to support pharmacists, allowing them to work at the top of their license. To facilitate these additional services, including clinical services, immunizations, tests to treat illnesses such as respiratory syncytial virus (RSV) and flu, pharmacists need connectivity and interoperability to bi-directionally receive and share data. The government cannot be responsible for these changes alone. Public and private partnerships must be created to drive a change in information sharing.

NCPDP is committed to continuing its role as a trusted partner and resource as HHS develops policies to improve health data exchange and promote innovation in consumer digital health products and enhance engagement among patients, caregivers and providers with digital healthcare solutions. NCPDP urges CMS and ASTP to recognize the essential role of pharmacists in supporting these efforts. NCPDP welcomes the opportunity to engage further, share our expertise and collaborate to make these goals a reality.

For direct inquiries or questions related to this letter, please contact:

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Respectfully,

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